CERVICAL CANCER

Clean disease factsheet | Overview, symptoms, diagnosis, treatment & prevention

1. Overview

Cervical cancer is a cancer arising from the cervix, the lower part of the uterus. Persistent infection with high-risk human papillomavirus (HPV) is the main cause. Most cervical cancers develop gradually from precancerous cellular changes, which can be detected and treated through screening.

2. Causes and Risk Factors

- Persistent infection with high-risk HPV types is the key cause.
- Risk is increased by smoking, immunosuppression, early sexual activity, multiple sexual partners, and limited access to screening.
- Lack of HPV vaccination and inadequate screening increase the risk of advanced disease.

3. Common Symptoms

- Abnormal vaginal bleeding, especially bleeding after sexual intercourse.
- Bleeding between periods or bleeding after menopause.
- Unusual or persistent vaginal discharge, sometimes with an unpleasant smell.
- Pelvic, lower abdominal, or back pain may occur, particularly in more advanced disease.
- Early cervical cancer may have no symptoms, which is why screening is important.

4. Diagnosis

- Screening can identify cervical precancer before symptoms develop.
- Abnormal screening results may require HPV testing, cytology and/or visual assessment.
- Suspected cancer is confirmed with appropriate clinical examination and biopsy.
- Imaging and other investigations may be used to determine the extent/stage of disease.

5. Treatment

- Treatment depends on the stage, tumour characteristics, overall health and fertility considerations.
- Early disease may be treated with surgery, including appropriate excisional procedures or hysterectomy depending on the case.
- Locally advanced disease is commonly treated with radiation therapy, often combined with chemotherapy.
- Advanced or recurrent disease may require systemic treatment, including chemotherapy and selected targeted/immunotherapy approaches.
- Supportive and palliative care should be offered when needed.

6. Prevention

- HPV vaccination is a major primary-prevention measure.
- Regular cervical cancer screening helps detect precancerous changes early.
- Avoiding tobacco and addressing other modifiable risk factors can reduce risk.
- Early evaluation of abnormal bleeding or discharge improves the chance of timely diagnosis.

Official reference sources
ICMR, Government of India — Cervix Cancer: Consensus Document for the Management of Cancer Cervix: https://www.icmr.gov.in/guidelines
Official ICMR PDF: https://www.icmr.gov.in/icmrobject/custom_data/pdf/resource-guidelines/Consensus%20Document%20for%20The%20Management%20of%20Cancer%20Cervix_0.pdf

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PNEUMONIA

Clean disease factsheet | Overview, symptoms, diagnosis, treatment & prevention

1. Overview

Pneumonia is an acute infection of the lungs in which the air sacs (alveoli) become inflamed and may fill with fluid or pus. It can be caused by bacteria, viruses, fungi or, less commonly, other organisms. Severity ranges from mild illness to life-threatening respiratory failure.

2. Causes and Risk Factors

- Common bacterial causes include Streptococcus pneumoniae and other bacteria; viral causes include influenza and other respiratory viruses.
- Risk is higher in young children, older adults and people with chronic diseases or weakened immunity.
- Smoking, malnutrition, overcrowding and delayed access to healthcare can increase risk or severity.

3. Common Symptoms

- Cough, sometimes with sputum.
- Fever, chills or feeling unwell.
- Shortness of breath or difficulty breathing.
- Chest pain, especially with deep breathing or coughing.
- Fast breathing, low oxygen, confusion or severe weakness can indicate serious illness.

4. Diagnosis

- Diagnosis is based on history and clinical examination, including respiratory rate and oxygen saturation.
- Chest imaging may be used when clinically indicated.
- Laboratory or microbiological testing may be required in severe disease or selected patients.
- Severity assessment helps determine whether home care, hospital care or intensive care is required.

5. Treatment

- Treatment depends on the likely cause and severity.
- Bacterial pneumonia may require appropriate antibiotics prescribed by a clinician.
- Viral pneumonia is generally managed with supportive care; specific antiviral treatment may be indicated for selected infections such as influenza.
- Oxygen therapy and hospital care may be needed when oxygen levels are low or breathing is severely impaired.
- Fluids, fever control, nutrition and treatment of complications are important supportive measures.

6. Prevention

- Vaccination against relevant respiratory infections can reduce risk of pneumonia and complications.
- Hand hygiene and respiratory etiquette reduce spread of respiratory infections.
- Avoiding tobacco smoke and improving nutrition support lung health.
- Prompt assessment of severe breathing difficulty is important.

7. Seek Urgent Medical Care If

- There is severe difficulty breathing, bluish lips/face, confusion, inability to stay awake, severe chest pain, or markedly low oxygen saturation.
- A child has fast or difficult breathing, inability to drink/feed, unusual sleepiness, or chest indrawing.

Official reference sources
ICMR, Government of India — Standard Treatment Workflows, including Severe Pneumonia: https://www.icmr.gov.in/standard-treatment-workflows-stws
ICMR Severe Pneumonia STW: https://www.icmr.gov.in/icmrobject/uploads/STWs/1725952336_paediatrics_severe_pneumonia.pdf
ICMR Standard Treatment Workflow for Acute Respiratory Infection in Adults: https://www.icmr.gov.in/icmrobject/custom_data/pdf/downloadable-books/STW_Manual_v1.pdf

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MEASLES

Clean disease factsheet | Overview, symptoms, complications, treatment & prevention

1. Overview

Measles is a highly contagious viral disease caused by measles virus. It spreads mainly through respiratory droplets and airborne particles from an infected person. It commonly causes fever and a widespread rash and can lead to serious complications, especially in young children and people with poor nutrition or weakened immunity.

2. Transmission and Incubation

- The virus spreads when an infected person coughs, sneezes or breathes out infectious particles.
- Symptoms commonly appear about 10–14 days after exposure, although timing can vary.
- Measles is highly contagious, so suspected cases should follow local public-health and infection-control advice.

3. Common Symptoms

- High fever.
- Cough.
- Runny nose (coryza).
- Red, watery eyes (conjunctivitis).
- A maculopapular rash that typically begins on the face/head and spreads downward.
- Small white spots inside the mouth (Koplik spots) may occur before the rash.

4. Complications

- Otitis media and diarrhoea can occur.
- Pneumonia is an important complication.
- Encephalitis can occur and may be severe.
- Severe disease and death are more likely in young children, malnourished people and those with weakened immunity.

5. Diagnosis

- Diagnosis is based on the clinical presentation and exposure/outbreak history.
- Laboratory confirmation may be performed using appropriate serology and/or molecular testing according to public-health protocols.
- Suspected cases should be reported and managed according to local public-health requirements.

6. Treatment

- There is no routine specific antiviral cure for uncomplicated measles; treatment is mainly supportive.
- Maintain hydration and provide appropriate nutrition.
- Fever and discomfort can be managed with appropriate medicines.
- Vitamin A is recommended for children with measles according to national/clinical guidance, particularly where deficiency is a concern.
- Bacterial complications such as pneumonia or otitis media may require specific treatment when diagnosed.

7. Prevention

- Vaccination with measles-containing vaccine is the most effective prevention.
- During suspected infection, reduce exposure to others and follow public-health/isolation advice.
- Good nutrition and timely vaccination help reduce severe disease.

Official reference sources
MoHFW, Government of India — National Operational Guidelines for Introduction of Measles-Rubella Vaccine: https://www.mohfw.gov.in/sites/default/files/Measles%20rubella%20vaccine%20operational%20guidelines.pdf
IDSP, MoHFW — Measles case definition: https://idsp.mohfw.gov.in/WriteReadData/l892s/4216524651671001411.pdf

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RABIES

Clean disease factsheet | Overview, symptoms, exposure management, treatment & prevention

1. Overview

Rabies is a viral zoonotic disease that affects the central nervous system. It is usually transmitted through the saliva of an infected animal, most often through a bite. Once clinical rabies develops, it is almost always fatal; however, rabies can be prevented after exposure by prompt and appropriate post-exposure prophylaxis (PEP).

2. Transmission and Exposure

- Transmission most commonly occurs through bites from an infected animal.
- Saliva contacting broken skin or mucous membranes can also constitute an exposure.
- Dogs are a major source of human rabies in many settings, but other mammals can transmit the virus.
- A suspected exposure requires prompt medical assessment; do not wait for symptoms.

3. Symptoms

- Early symptoms may include fever, headache, malaise, sore throat and abnormal sensations such as pain, tingling or numbness at the bite site.
- Neurologic disease can progress to agitation, restlessness, tremors and painful spasms.
- Hydrophobia (difficulty/fear associated with drinking water) is a characteristic feature of many cases.
- Aerophobia, confusion, hallucinations, seizures or paralysis may occur.
- Progression can lead to coma and death.

4. Diagnosis

- Diagnosis is based on clinical features, exposure history and specialized laboratory testing.
- Laboratory confirmation may use tests on appropriate clinical specimens in specialized laboratories.
- Public-health and laboratory protocols should be followed for suspected cases.

5. Treatment After an Animal Exposure (PEP)

- Immediately wash and flush the wound thoroughly with soap and running water.
- Seek urgent medical care for a formal exposure-risk assessment.
- Rabies vaccine is given according to the recommended post-exposure schedule when indicated.
- Rabies immunoglobulin is indicated for eligible category III exposures and should be administered according to clinical guidance.
- Do not apply irritant substances to the wound and do not delay medical evaluation.

6. Treatment of Clinical Rabies

- Once clinical rabies develops, there is no reliably effective curative treatment.
- Management is primarily supportive and may require intensive care.
- Prevention through vaccination and timely PEP after exposure is therefore critical.

7. Prevention

- Vaccinate dogs and other eligible animals through public-health programmes.
- Avoid approaching unfamiliar or stray animals and teach children safe animal behaviour.

- Consider pre-exposure vaccination for people with occupational or other high-risk exposure.
- After any potentially rabid animal exposure, start wound care and seek medical assessment immediately.

Official reference sources
NCDC, MoHFW, Government of India — Rabies: General Aspects & Laboratory Diagnostic Techniques (2022): https://ncdc.mohfw.gov.in/wp-content/uploads/2024/04/RabiesGeneralAspectsLaboratoryDiagnosticTechniques2022.pdf
NCDC Technical Guidelines — National Guidelines for Rabies Prophylaxis: https://ncdc.mohfw.gov.in/includes/Resource_Library/index.php?tab=Technical+Guidelines
MoHFW Clinical Establishments Division — National Rabies Control Programme: https://clinicalestablishments.mohfw.gov.in/

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